



WOMEN'S HEALTH, GENETICS, MENTAL HEALTH DIRECTORATE CLINICAL PRACTICE GUIDELINE	
Mental Health Emergency, Management of	
Scope (Staff):	KEMH WNHS staff caring for women with mental illness
Scope (Area):	King Edward Memorial Hospital
This document should be read in conjunction with the Disclaimer .	

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1. Aim

The purpose of this guideline is to ensure consistency and best practice for the management of mental health emergencies related to Acute health-related Behavioural Disturbance (ABD).

This guideline applies to:

- Patients accessing inpatient and outpatient services in the Women and Newborn Health Service (WNHS) at King Edward Memorial Hospital who are displaying ABD.

This guideline:

- Highlights key concerns in the care and treatment of ABD.
- Provides staff with clear pathways and guidelines for the management of ABD.
- Provides staff with advice on the safe use of medication and/or physical restraints in the management of ABD.
- Prevents potential harm to patients and/or fetus/baby, the public and staff.
- Minimises the need for more invasive interventions such as chemical and/or physical restraint.
- Ensures all clinical staff practice in accordance with the [Mental Health Act 2014](#) (MHA 2014) and consistent with [Department of Health MP 0181/24 Safety Planning for Mental Health Consumers Policy](#), [Department of Health MP 0186/24 Use of Restrictive Practices in Non-Authorised Healthcare Settings Policy](#) as well as the [MP 0159/21 Workplace Aggression and Violence Policy](#) and the [NMHS Stop the Violence Strategy 2020-2025](#).

This guideline does **not** apply to:

- Visitors to the hospital.

2. Background

The development of this Clinical Practice Guideline resulted from an identified gap in policy guiding staff with clear pathways of management of ABD.

3. Risk

Non-compliance with this policy will:

Breach legislative requirements	☐	Impact on Patient Quality of Care	☒
Breach National/State/Mandatory Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☐	Impact financial operations	☐
Damage NMHS reputation	☒	Impact on Staff Safety	☒
Other	☐		

4. Key Points

- In keeping with the [NMHS Stop the Violence Strategy 2020-2025](#), WNHS is committed to the provision of a safe environment for staff, patients and visitors.
- Early recognition and using supported de-escalation skills enables appropriate and reasonable responses to ABD.

- Clinicians responding to ABD must exhibit recovery-oriented, person-centred, trauma-informed, culturally competent, and developmentally appropriate care values.
- Possible causes of agitation include:
 - Delirium
 - Organic brain syndromes
 - Acute alcohol/drug intoxication/withdrawal
 - Acute anxiety
 - Trauma response
 - Psychotic disorders
- Identify a clear Management of Mental Health Emergency Flowchart ([Appendix 1](#)).
- Assess level of arousal using the Agitation and Arousal Rating Scale ([Appendix 2](#)).
- Capacity is time and task specific and must always be assessed in the context of the decision that is made. All adults are presumed to have capacity unless there is evidence to suggest otherwise.
- If a patient is deemed to have capacity, they are entitled to make decisions which others may not agree with or deem to be unwise.
- Consent to treatment is a person's agreement that a health professional can proceed to perform a specific treatment or procedure.
- A patient can be treated without consent if they refuse treatment or are unable to consent because of their mental state, and treatment is required to:
 - to save the person's life; or
 - to prevent the person from behaving in a way that is likely to result in serious physical injury to the person or another person ([MHA 2014 s202\(1\)](#)).
 - This applies to all patients.
- Failure to resist treatment does not constitute consent to treatment, and clinicians must consider a patient's wishes and if practical, seek consent for treatment.
- The doctrine of necessity may provide protection from clinical and/or civil liability for the very short-term use of restraint (and detention) in situations akin to an emergency. The common law doctrine of necessity only authorises restraint and detention where:
 - The restraint was done to avoid an imminent peril to life or serious injury;
 - The person carrying out the restraint believed the individual was placing him or herself or others in a situation of imminent peril; and
 - The restraint was proportionate to the potential harm that could have occurred.

- The medical practitioner providing Emergency Psychiatric Treatment (EPT) under the [Mental Health Act 2014](#) must, as soon as is practicable, complete a Form 9A.
- Clinicians must practise the least harmful, least restrictive approach prior to using physical restraint and/or intramuscular/intravenous sedation.
- Medication should only be used as an adjunct to other interventions (e.g., behavioural interventions).
- Physical restraint should only be carried out by appropriately trained staff and in accordance with [MP 0186/24 Use of Restrictive Practices in Non-Authorised Healthcare Settings Policy](#).
- Physical restraint should only be used when all reasonable steps have been taken to avoid it, as per [Chief Psychiatrist's Standards for Clinical Care](#) and [Applying the Mental Health Act 2014 in WNHS guideline](#).
- Maintain any physical restraint for the shortest time possible; always aim for the least restrictive intervention; and assess risk and restraint dynamically.
- If restraint is applied to a patient in the MBU to provide emergency treatment, [Mental Health Act Form 10 \(Restraint\)](#) must be completed (10A to I, as appropriate). This form is not required for restraint outside the MBU ([MHA 2014](#) s230, 231).
- Consider the patient's obstetric condition, co-morbid medical illness, substance use, and potential re-traumatisation.
- The prone position is associated with significant medical risks (depression of ventilation, asphyxia, musculoskeletal injuries, and potential cardiac arrest) and should therefore be maintained for a maximum of three minutes with continuous monitoring of the patient's respiratory condition. It should be avoided in pregnant patients.
- De-escalation and restraint must be supported with regular and mandatory training for staff in the management of aggression.

5. Process

Refer to [Appendix 1](#): Management of Mental Health Emergency Flowchart.

6. De-escalation

The following are general points and strategies for de-escalation:

- Where possible, identify the cause of agitation.
- Environmental influences
 - Is the environment overstimulating? Consider relocating the patient to a quieter, lower-stimulus environment, if possible, away from other patients and bystanders.
 - Are there environmental hazards or dangerous items that need to be removed?

- Minimise the likelihood of the patient absconding.
- Non-verbal strategies (what do I do?)
 - Take a deep breath.
 - Approach the patient in a calm, confident and non-threatening manner, where they can see your hands at all times.
 - Never approach a patient who is holding or has access to a potential weapon.
 - Keep a safe distance and know your points of egress.
 - Know where your fixed wall duress buttons are located.

Sit down if safe to do so

- Your facial expressions, manner and tone of voice convey more meaning than words.
 - Involve trusted relatives, support person, or staff when talking to the patient to improve rapport.
- Verbal strategies (what do I say?).
 - Introduce yourself, your role, and outline any purpose of conversations.
 - Use the client's name to attract and retain their attention.
 - Use clear, simple language (not medical jargon).
 - Be empathic, non-judgmental, and respectful. Listen and acknowledge the patient's concerns, needs, or fears, and emphasise your desire to help.
 - Validate their feelings, encourage positive behaviours, and emphasise desirable outcomes.
 - Offer an apology (if appropriate).
 - Avoid potentially provocative statements such as, "calm down", or issue threats as this may escalate their behaviour.

7. Assessment and Management of ABD

Mental health and medical professionals should use the following guidance for assessment and management of ABD:

- Seek documentation of past and present psychiatric, medical, and pregnancy histories of patients prior to admission to identify risk factors for ABD.
- If possible, access Psychiatric Services Online Information System (PSOLIS) to identify previous alerts and risks related to ABD.
- Where possible, identify the cause of agitation.
- Reassess as necessary, especially in situations of changing mental or physical presentation.

- Assess for the presence, and extent, of mental health condition/s according to arousal levels in three categories, mild (arousal levels 1-2), moderate (arousal levels 3-4) and severe (arousal level 5).
- Include a safety plan in the clinical file of all patients with mental health issues who have a risk of becoming violent or aggressive (please refer to [Safety Planning for Mental Health Consumers Policy](#)).
 - If a patient is displaying ABD that is related to known mental health concerns or unknown conditions, contact duty (business hours) on 6458 1521 or on-call (out of hours) on 6458 1623, psychiatry registrar and/or consultant psychiatrist for assessment and management advice.
 - Apply duty of care principles while awaiting psychiatric assessment.
 - If there is imminent threat to staff, visitors, or other patients, dial 55 – Code Black.
 - If required, access the Employee Assistance Programme for additional support following the incident. See [WNHS Supporting Our Staff Hub](#) and [NMHS Employee Assistance Program Hub](#) for further information.

7.1. Mild to Moderate Arousal (Levels 1 – 4): Requiring Light to Conscious Sedation

- Refer to Appendix 2
- Light to conscious sedation can be administered by any medical practitioner, including the on-call or duty psychiatrist.
- If the potential for ABD has been identified prior to admission, document the possible need for medication (e.g., in the complex care plan).
- Use oral medication where possible (levels 2-4); only administering intramuscular medication as a last resort when risk assessment deems necessary (level 4).
- Carefully consider the risks and benefits of prescribing medication.
- Where possible, explain and discuss medication regimes with the patient. Consider advance health directives, personal choice, and post-birth decisions such as breastfeeding.
- Ensure medication plans maintain continuity through birth into the postpartum period.
- For pregnant women, use antipsychotics or benzodiazepines with a short half-life or at the minimum effective dose to reduce extrapyramidal symptoms or the risk of floppy baby syndrome, respectively¹.
- In pregnant patients requiring restraint, administer intramuscular medication with the patient positioned in a semi-recumbent position, left lateral position or using a pelvic wedge.

¹ NICE Guideline, *Antenatal and Postnatal Mental Health: Clinical Management and Service Guidance* (p. 830)

- All medication administered must be recorded on the [Agitation and Arousal PRN Medication Chart \(MR 810.09\)](#).

7.2. Severe Arousal (Level 5): Requiring Deeper Sedation

- Treatment should be provided as per the [Guideline for safe care for patients sedated in health care facilities for acute behavioural disturbance \(Joint guideline RANCP, ACEM, ANZCA, and CICM\)](#).
- Only clinicians trained in recognising and managing an obstructed airway and trained in maintaining oxygenation, ventilation and cardiovascular support should administer medication for conscious and deeper sedation.
- This team should include:
 - a medical practitioner skilled in cardiopulmonary resuscitation, airway management and advanced life support. This will usually be the anaesthetist on the Code Blue team.
 - at least one nursing staff member who is trained in Basic Life Support/Advanced Life Support and who is competent to monitor the patient and to assist the medical practitioner administering the sedation.
 - at least one additional trained staff member to assist as required with disturbed behaviour occurring during sedation. This will usually be the on-call psychiatrist.
- Monitor circulation, ventilation, and oxygenation at least every 30 minutes.
- If extending sedation, either in duration or depth, the attending medical practitioner/s should undertake the following monitoring:
 - Consciousness – use of sedation scales is recommended.
 - Extrapyrimal side effects.
 - Mental state.
 - Urinary output to detect urinary retention.
 - Hydration.
 - Pressure care.
 - Venous Thromboembolism (VTE) prophylaxis consideration.
- In pregnant women consider the need for additional monitoring. Consult with relevant medical personnel (obstetrician, anaesthetist, physician and paediatrician) as required.
- All medication administered must be recorded on the [Agitation and Arousal PRN Medication Chart \(MR 810.09\)](#).

8. Sedation and Medication Management

- Conscious sedation and deeper sedation form a continuum. Deeper sedation may occur unintentionally in the context of ABD, due to the individual variable response to medications and the narrow therapeutic index of sedating agents.
- For both oral (PO) medications and intramuscular injections (IM), responsiveness must be measured and recorded at baseline and at 30 to 60 minutes after administration and repeated as necessary.
- Measure and record responsiveness on Appendix 3: Sedation Assessment Tool (SAT).

9. Capacity Assessment

- All adults are presumed to have capacity unless there is evidence to suggest otherwise.
- Assessing a patient's capacity to consent to medical treatment remains the responsibility of the treating team.
- A patient is presumed to have capacity based on the following factors:
 - Understanding the information provided. The choice which a patient makes will be based on the amount and quality of information provided to them in a manner they can understand. Consider the use of audio-visual aids, information sheets, interpreter services, and involving trusted family/carers to support decision making.
 - Retaining the relevant information required for them to make a decision. The patient needs to be given sufficient time to consider their decision.
 - Appreciation of the consequences for them in making their decision.
 - Communication of their choice through a logical process and in a consistent manner.
 - If a patient is deemed to have capacity, they are entitled to make decisions which others may not agree with or deem to be unwise.

10. Guidance for Pregnancy and Postpartum Restraint

- Physical restraint should only be used when all reasonable steps have been taken to avoid its use, as per [Chief Psychiatrist's Standards for Clinical Care](#) and [Applying the Mental Health Act 2014 in WNHS guideline](#).
- Physical restraint should only be carried out by appropriately trained staff.

11. Definitions

Mental Health Emergency	A situation within the context of health care where a patient requires clinical management for an Acute health-related Behavioural Disturbance (ABD).
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Acute health-related Behavioural Disturbance (ABD)	Combined physical actions of an individual which are considered excessive in context and may result in significant harm to the individual, others, or property. ABD is characterised by a rapid onset and severe intensity. Often the individual is not considered to be in voluntary, or legally competent, control of their behaviour. In this guideline, ABD is associated with, or caused by: – A mental disorder – Medical conditions or physical illness, or – Intoxication with alcohol and/or other substances.²
Light sedation	A drug-induced state in which patient has a normal response to verbal stimulation; their airway, ventilation and cardiovascular function is unaffected.
Conscious sedation	A drug-induced depression of consciousness during which the patient can respond purposefully to verbal commands or light tactile stimulation. Their cardiovascular function is normally maintained, and ventilation is adequate, but may require support.
Deeper sedation	A drug-induced patient state that can readily progress to loss of consciousness and where patient responds only to painful stimulation. It is associated with patient loss of ability to maintain airway, inadequate spontaneous ventilation and/or impaired cardiovascular function, and has similar risks to general anaesthesia, requiring an equivalent level of care.
Bodily restraint	The physical or mechanical restraint of a person who is being provided with treatment or care, to restrict that person's movement.
Psychiatric Services Online Information System (PSOLIS)	Centrally based mental health clinical information system developed for use by medical, nursing, allied health, management, and clerical staff within the public Mental Health Service. It provides clinicians with the ability to share, analyse and retrieve information about consumers.

Related policies, procedures and guidelines

[Department of Health MP 0186/24 Use of Restrictive Practices in Non-Authorised Healthcare Settings Policy](#)

[Department of Health MP 0181/24 Safety Planning for Mental Health Consumers Policy](#)

² ACEM, ANZCA, CICM, RACZCP. *Guidelines for Safe Care for Patients Sedated in Health Care Facilities for Acute Behavioural Disturbance*. 2019

Related policies, procedures and guidelines

[WNHS Emergency Management Code Black Procedures](#)

[Applying the Mental Health Act 2014 in WNHS Guideline](#)

[Department of Health MP 0159/21 Workplace Aggression and Violence Policy](#)

[Department of Health MP 0181/24 Safety Planning for Mental Health Consumers Policy](#)

[NMHS Mental Health Transport Hub](#)

[NMHS Mandatory Training Policy](#)

[Department of Health Guideline: Management of acute behavioural disturbance in COVID-19](#)

[Department of Health MP 0175/22 Consent to Treatment Policy](#)

References

Child and Adolescent Mental Health Service (2022) [Arousal and agitation management for mental health inpatients](#) Government of Western Australia

Rockingham Peel Group (2022) [Acutely Aroused Patient Clinical Practice Standard \(Clinical Practice Manual Code CPS: 006\)](#) South Metropolitan Health Service

Rockingham Peel Group (2023) [Full Neurological Observations - Adult Clinical Practice Standard \(Clinical Practice Manual Code CPS: 008\)](#) South Metropolitan Health Service

Fiona Stanley Fremantle Hospitals Group (2022) [Medication Management of Agitation & Arousal in Mental Health Inpatients Guideline](#) South Metropolitan Health Service

Fiona Stanley Fremantle Hospitals Group (2021) [Acute Severe Behavioural Disturbance \(ASBD\) Management in the Emergency Department Guideline](#) South Metropolitan Health Service

[Chief Psychiatrist's Guideline issued under Sections 547 \(3\) and 548 of the Mental Health Act 2014: Use of Detention and Restraint in Non-authorised Healthcare Settings](#)

Useful resources

[Guideline for safe care for patients sedated in health care facilities for acute behavioural disturbance](#)

[Medical Record Form: Agitation and Arousal PRN Medication Chart \(MR810.09\)](#)

[NMHS Stop the Violence Strategy 2020-2025](#)

[Clinical Practice Guide to the Mental Health Act 2014 3rd Edition](#)

[Chief Psychiatrist's Standards for Clinical Care](#)

[Mental Health Act 2014](#)

[NICE Guideline 10: Violence and aggression: short-term management in mental health, health and community settings](#)

Useful resources

[NICE Guideline Antenatal and Postnatal Mental Health: Clinical Management and Service Guidance](#)

[NICE Guideline Antenatal and Postnatal Mental Health: Overview](#)

[NMHS Employee Assistance Program](#)

[WNHS Supporting our Staff Resources](#)

For more information about pregnancy and restraint see:

[Clinical Practice Guide to the Mental Health Act 2014 3rd Edition \(Chapter 6: Treatment, Electroconvulsive Therapy, Psychosurgery, Mental Health Interventions and Urgent Non-Psychiatric Treatment\)](#)

[Chief Psychiatrist's Standards for Clinical Care Standard: Seclusion and Bodily Restraint Reduction](#) and [Reporting Episodes of Seclusion and Restraint](#)

[WA Department of Health Mental Health Act 2014 Forms 10A – 10I \(Restraint\)](#)

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Version History

Version Number	Date	Summary
1.0	February 2020	Initial version
1.1	March 2021	Minor amendment
1.2	May 2021	Change of best practice literature in therapeutic guidelines
1.3	September 2021	Updated link in ABD Inpatient Flowchart
1.4	February 2022	Amendment to policy sponsor
2.0	October 2024	Full review; update to new template; significant changes throughout guideline; added reference to Agitation and Arousal PRN Medication Chart medical

		record form; added references to new WA Health Mandatory Policies.
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The health impact upon Aboriginal people has been considered, and where relevant incorporated and appropriately addressed in the development of this policy.

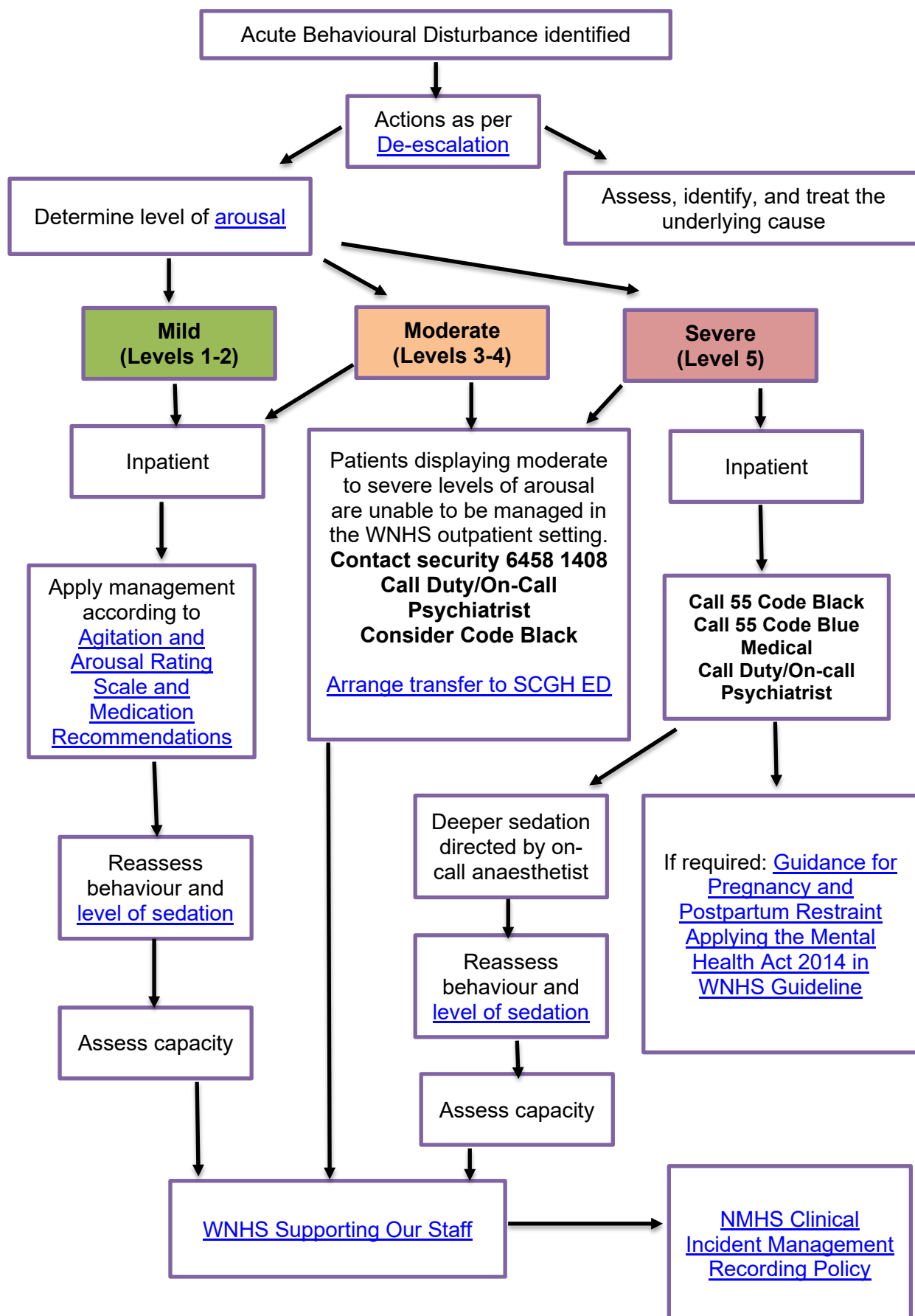
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Appendix 1: Management of Mental Health Emergency Flowchart



Appendix 2: Agitation and Arousal Rating Scale and Pharmacological Management Recommendations

Level 0	Level 1 (Mild)	Level 2 (Mild)	Level 3 (Moderate)	Level 4 (Moderate)	Level 5 (Severe)
Asleep or unconscious	Settled, minimal agitation	Mildly aroused, pacing, still willing to talk reasonably	Moderately aroused, agitated, becoming more vocal and unreasonable or hostile	Highly aroused, possibly distressed, fearful, possibly noisy but not violent	Highly aroused, violent to self, others, property, refusing oral medication
Review medication requirements Refer to Sedation Assessment Tool	PRN not required	Lorazepam 0.5 to 1 mg orally OR Quetiapine IR 12.5 to 25 mg orally OR Olanzapine 2.5 mg orally or subling	Lorazepam 1 to 2 mg orally OR Quetiapine IR 25 to 50 mg orally OR Olanzapine 5 mg orally or subling	Lorazepam 1 to 2 mg orally AND Quetiapine IR 25 to 50 mg orally	Midazolam 2.5 to 5 mg IM OR Lorazepam 0.5 to 1 mg IM OR Olanzapine 5 mg IM Seek anaesthetics support

Medication	Oral	Intramuscular
Lorazepam	Repeat every 2 to 6 hours up to 4 mg in 24 hrs	Repeat after 20 minutes up to 4 mg in 24 hrs
Quetiapine	Repeat 2 to 6 hours up to 400 mg in 24 hrs	N/A
Olanzapine	Repeat 2 to 6 hours up to 30 mg in 24 hrs	Repeat 2 to 6 hours up to 30 mg in 24 hours
Midazolam	N/A	Repeat after 20 minutes up to 10 mg in 24 hours

Medications should only be used as an adjunct to de-escalation strategies.

Specify exact dose – do not use dosing ranges. **When prescribing clearly specify the level of agitation that is being treated (e.g. Lorazepam 0.5mg for Level 2 arousal).**

Where possible, select medications that are the same or similar to the patient's regular medications.

Benzodiazepines are the drugs of first choice in tranquilisation and have fewer adverse effects than antipsychotics. Ensure flumazenil is available if administered parenterally. QT prolongation, acute dystonia, and neuroleptic malignant syndrome are associated with antipsychotics.

Do not give IM benzodiazepines within 1h of IM olanzapine.

Measure and record responsiveness at baseline and at 30-60 minutes after administration. Repeat as necessary.

Appendix 3: Sedation Assessment Tool

Sedation Assessment Tool (SAT)		
Score	Responsiveness	Speech
+3	Combative, violent, out of control	Continual loud outbursts
+2	Very anxious and agitated	Loud outbursts
+1	Anxious and restless	Normal/ Talkative
0	Responds easily to name	Speaks normally
-1	Asleep but rouses if name is called	Slurring or prominent slowing
-2	Physical stimulation	Few recognisable words
-3	No response to stimulation	Nil